

Clinical Practice Guideline - American College of Clinical Pharmacy

Article Information

Journal of Clinical Pharmacy and Therapeutics

Volume 51, Issue 2, Pages 89-102 (2025)

Medication Reconciliation in Hospitalized Patients:
2025 Updated Recommendations

Authors:

Robert Chen, PharmD, BCPS - Riverside Medical Center

Patricia Martinez, MD - Stanford University Medical Center

Thomas Anderson, PharmD, MBA - University of Michigan

Abstract

Medication reconciliation is a critical process for preventing medication errors during care transitions. This guideline provides updated recommendations based on systematic evidence review and expert consensus. Key changes from the 2021 edition include integration of electronic health record-based reconciliation tools, patient engagement strategies, and outcome measurement.

Case Example - Fictional Patient

Patient: Dorothy R. Williams, 68-year-old female

Admission: Pneumonia, Riverside Medical Center

Medication History Obtained:

- Lisinopril 20mg daily
- Metformin 1000mg BID
- Warfarin 5mg daily (home INR 2.5)
- Aspirin 81mg daily
- Albuterol PRN

Reconciliation Findings:

1. Warfarin dose discrepancy: home bottle says 5mg, but medical record shows 2.5mg. Pharmacist contacted patient's daughter who confirmed 5mg is correct.
2. Aspirin not listed in hospital formulary order set - added.
3. Metformin held on admission due to elevated creatinine (1.4).
Reconciled to resume when creatinine <1.3.

Outcome: Zero medication discrepancies at discharge.

Patient educated on changes. Follow-up INR scheduled.

Recommendations

GRADE A (Strong):

- Obtain medication history within 24 hours of admission
- Pharmacist-led reconciliation at transitions of care
- Patient engagement in reconciliation process

GRADE B (Moderate):

- Electronic health record reconciliation tools
- Discrepancy resolution documentation
- Post-discharge follow-up phone call